

CHAPTER

7



Visuchika, Alasaka and Vilambika

Learning Objectives

- ⇒ To understand the concepts of Visuchika, Alasaka and Vilambika
- ⇒ To understand the Nidana Panchaka of Visuchika, Alasaka and Vilambika mentioned in Ayurveda literatures
- ⇒ To frame out the probable Samprapti Ghataka based on the progression of the diseases
- ⇒ To describe Lakshana, Upadrava, Sadhyasadyata of Visuchika, Alasaka and Vilambika

Introduction

Visuchika, Alasaka and Vilambika are the set of the diseases which affect the *Annavaha Srotas*. Alasaka and Visuchika are together termed as *Ama Pradosha* as per Charaka. These are acute clinical manifestations of *Annavaha Srotas*. These diseases are usually of shorter duration and these are the outcomes of improper management of *Amajirna*, *Vishtabdhajirna* and *Vidagdhajirna*. This information indicates that the descriptions about *Nidana*, *Samprapti*, *Lakshana* etc mentioned in the context of *Ajirna* can be incorporated in the context of *Visuchika*, *Alasaka* and *Vilambika*. In other words, these diseases are the *Upadrava* of *Ajirna* where in there is abnormal vitiation of *Kapha*, *Vata* and *Pitta* respectively.

अजीर्णमामं विग्धं विदग्धं च यदीरितम्।

विसूच्यखसकी तस्माद्भवेच्चापि विलम्बिका॥1 5॥ Ma.Ni. – 6/15

Madhava Nidana states the causes of *Visuchika*, *Alasaka* and *Vilambika* are *Amajirna*, *Vishtabdhajirna* and *Vidagdhajirna* respectively.

Visuchika¹

सूचीमिरिच गात्राणि तुदन् संतिष्ठतेऽनिखः।

यत्राजीर्णेन सा वैचैर्विपीति निगद्यते॥1 6॥

न तां परिमिताहारा लभन्ते विदितागमाः।

सूदास्तामजितात्मानो लभन्तेऽशनलोलुपः॥1 7॥ Ma.Ni. – 6/16,17

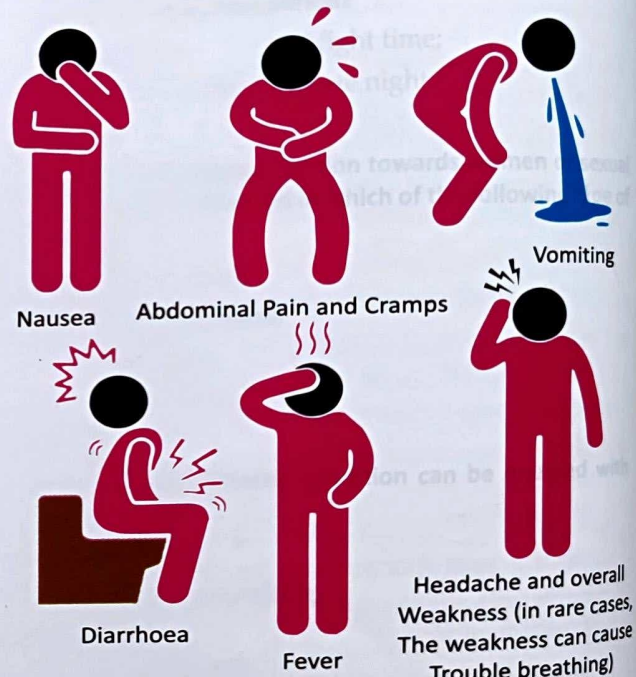


Figure 11: Lakshana of Visuchika

‘विविधेर्वेदनाभेदेबोव्यादेभ्रेशको पतः।

सूचीनिरिच गात्राणि भिनत्तीति विसूचिका’

Madhu, Ma.Ni. – 6/16,17

*Visuchika*² is a condition where in an individual observe himself regarding pricking type of pain all over the body.

1 Figure 11: Lakshana of Visuchika

2 Visuchika is a *Annavaha Srotovikara* and the cardinal feature of Visuchika is not related to *Annavaha Srotovikara*. How it is possible?
 • To answer this question, first we have to identify what are the cardinal features of Visuchika. Upward and downward movement of vitiated Dosha along with excess fluid is the main reason for abnormal fluid loss. This fluid loss is associated with electrolyte imbalances in terms of sodium, potassium, chloride and calcium ions. These electrolytes are required for every muscular and neurological activity

This is considered to be due to dominance of *Kapha* and *Vata Dosha* in the body. The dominance of *Kapha Dosha* is manifested by *Amajirna* which occurs due to the abnormal vitiation of *Kapha Dosha* in *Mahasrotas* (alimentary canal). As per *Madhava Nidana*, the person who is not consuming the food without taking considering appropriate dietary measures.

Lakshana of Visuchika

मुच्छोऽतिसारो मधुः पिपासा शूलो प्रभोद्वेष्टनजृम्भदाहाः।

वैवकम्पी हृदये रुजन्म भवन्ति तस्यां शिरसः॥१८॥ Ma.Ni. – 6/18

✿ **Murcha³** – altered state of consciousness

✿ **Atisara⁴** – frequent passage of liquid or semi liquid stools

✿ **Vamathu⁵** – vomiting

✿ **Pipasa⁶** – thirst

✿ **Shula⁷** – pricking type of pain the abdominal region

✿ **Bhrama⁸** – giddiness

✿ **Udveshtana⁹** – muscle cramps in the calf region

✿ **Jrimbha¹⁰** – excess yawning

✿ **Daha¹¹** – burning sensation

✿ **Vaivarnya¹²** – discolouration

✿ **Kampa¹³** – involuntary movements

✿ **Hridaya Ruja¹⁴** – chest discomfort

✿ **Shirasah Bheda¹⁵** – tearing type of pain in the head region

in the body.

- When the person is having excess fluid from two of his major orifices i.e. mouth and anus, the person is usually avoiding the fluid intake only. This will lead to the depletion of water, sodium, potassium, calcium ions from the body leading to electrolyte imbalance which may have generalised clinical manifestation. Fi
- This leads to the cardinal feature of Visuchika i.e. pricking type of pain all over the body.
- 3 This is attributable to excess fluid loss from the body. As we all know the percentage of water in the human body is about 2/3rd of the human body (66.6%) and when an individual is presented with excess fluid loss from the body like diarrhoea or vomiting, there will be shrinkage of intravascular, interstitial and intracellular compartments. The fluid concentration is profoundly reduced so that the brain receive minimum amount of blood supply. This may lead to altered state of consciousness which may range from delirium to coma.
- 4 This is one of the cardinal feature of Visuchika. In this case, the person will have loss of concentration of fluid from body. During this clinical presentation, the person will have expulsion of more amount of glucose, sodium and potassium ions from the body. These essential nutrients such as sodium, potassium, glucose and water will be lost from the body. Sodium and potassium ions are required for every activity of cell skeletal muscle contraction, smooth muscle activity, cardiac muscular activity and neuro connections too.
- 5 In this evaluation of this symptom too, the person will have considerable amount of loss of water, glucose, sodium and potassium. The pathological events occur in this situation is like pathological aspects of the king.
- 6 As per current understanding, the term Trishna refers to thirst. When an individual presents with thirst, it indicates considerable amount of fluid loss from the body. Gastrointestinal and pharyngeal stimuli influence mechanism of thirst. Drinking of water in such occasion can have temporary relief from thirst. Gastro intestinal distension can partially alleviate thirst. After a person drinks water, 30 to 60 minutes may be required for the water to be reabsorbed and distributed throughout the body and then only the person may have proper restoration of blood volume to be normal and thirst sensation will be relieved.
- 7 This is attributable to the inflammatory reaction occurring in the alimentary canal. This inflammation reaction may occur in stomach, small intestine and large intestine.
- 8 This clinical presentation can occur in patients who are having dehydration where syncopal episodes are observed due to cerebral hypoperfusion.
- 9 This can be attributable to electrolyte imbalance, hyponatremia and/or hypokalemia in particular. This can occur in any part of the body, calf muscles, abdominal muscles, skeletal muscles of the upper and lower limb.
- 10 Yawning is an activity consists of wide opening of mouth with maximal widening of jaw, together with a long and deep inhalation through mouth an nose, followed by slow expiration, associated with a feeling of comfort. The average duration of yawn is 5 seconds. Additionally, stretching of the limbs also frequently accompanies yawning in humans. Following hypotheses can be put forth in understanding the concept of yawning.
 - When an individual does yawning, that will lead to the shift of temperature from the core of the head region to the periphery of head and neck.
 - Yawning relieves the pain in the ears by opening of eustachian tube which has a connection with middle ear by removing the blockage present in the tube.

Yawning is triggered when blood or brain oxygenation is insufficient, when O₂ level is decreased and CO₂ is increased. Yawning removes CO₂ from the lungs and increases the O₂ circulation in the brain.
- 11 This can be attributable to nutrition deficiency.
- 12 As per Ayurveda, the Varna includes not only colour, it also includes texture of the skin. In this occasion, the term Vaivarnya refers to abnormal loss of skin turgor.
- 13 This can be attributable to electrolyte imbalance, hyponatremia and/or hypokalemia in particular. Sometimes, hypocalcemia may also lead to involuntary movements.
- 14 This can be attributable to hypovolemic shock where cardiac compromise has occurred.
- 15 This can be attributable to cerebral hypoperfusion which can occur as a result of hypotension which results due to the shrinkage of intravascular compartment.

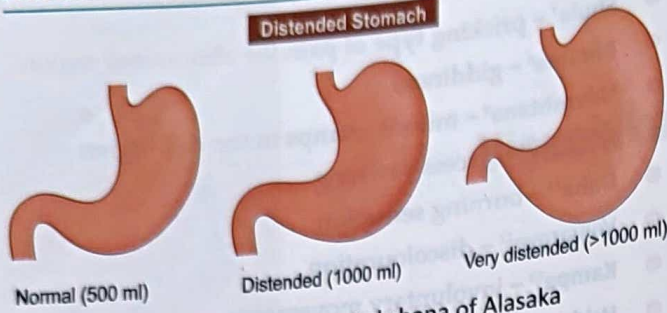
Alasaka¹⁶

Figure 12: Pratyatma Lakshana of Alasaka

कुक्षिरानातेऽत्यर्थं प्रताप्येत्परिकूजति।
निरुद्धो मारुतचैव कुक्षावुपरि धावति॥१॥

बातवचनिरोधश्च यस्यात्यर्थं भवेदपि।

तस्यालसकमाचष्टे तुष्णोद्गारी च यस्य तु॥२०॥ Ma.Ni. - 6/19,20

'प्रयाति नोर्ध्वं नाधस्तादाहारो न विपच्यते।

आमाशयेऽलसीभूतस्तेन चालसकः स्मृतः' Madhu, Ma.Ni. - 6/19,20

In *Alasaka*¹⁷, the abdominal region¹⁸ is distended excessively and the person suffering from this condition will be presented with darkness in front of the eyes and makes cooing type of sounds from the mouth. Downward movement of *Vata Dosha* is becoming reversed and associated with excess retention of flatus and faecal matter, thirst and absence of eructation.

Vilambika

दुष्टं तु भुक्तं कफमारुताभ्यां प्रवर्तते नोर्ध्वमधश्च यस्य।
विलम्बिकां तां भृशदुश्चिकित्स्यामाचक्षते शास्त्रविदः पुराणाः॥२१॥

Ma.Ni. - 6/21

The disease, *Vilambika* is manifested in an individual due to the vitiation of *Pitta* dominant *Ajirna* i.e. *Vidagdhaajirna* if timely management is not administered to the suffering individual. *Vilambika* is the disease where there will be features similar to *Alasaka* are seen i.e. non movement of vitiated *Dosha* from oral and anal routes. This is manifested in an individual due to the excess vitiation of *Kapha* and *Vata Dosha*. As far as the management of the

disease is concerned, it is very difficult to cure. Due to the exposure to the etiological factors, first there will be poor functioning of *Agni*. This stage leads to the formation of *Ama* in the *Amashaya*. If the person is not following appropriate measures, in this situation, that will lead to the vitiation of *Vata* and *Kapha Dosha*. This stage of the disease will further hampers the function of *Agni* (digestive fire) and further leads to the formation of more amount of *Ama*.

Food gets stagnated and remain in the state of indigestion. This leads to the fermentation of ingested food and it will lead to the formation of *Amavisha* (toxin) and leads to the manifestation of *Ajirna*.

Dandalasaka is another name of Vilambika

Difference between *Alasaka* and *Vilambika*: When we learn the concepts of *Alasaka* and *Vilambika*, both are considered to be manifested due to the vitiation of *Kapha* and *Vata Dosha*. The main difference between the two is severity of pain. In *Alasaka*, the person will be presented with severe pain in the abdominal region, where as in the patient of *Vilambika*, the person will have mild pain as the person will having the disease is chronic in duration.

Asadhya Lakshana of Visuchika and Alasaka

यः श्यावदन्तौष्ठनखोऽल्पसंज्ञो बभ्रुर्दितोऽभ्यन्तरयातनेत्रः।
क्षामस्वरः सर्वविमुक्तसन्धिर्यायात्ररः सोऽपुनरागमाय॥२३॥

Ma.Ni. - 6/23

Asadhya Lakshana¹⁹ of Visuchika and Alasaka include

- Blackish discolouration of teeth, lips and nails
- Altered state of consciousness
- Vomiting
- Sunken eyes appearance
- Feeble voice
- Looseness of all joints

16 Figure 12: Pratyatma Lakshana of Alasaka

17 In *Alasaka*, the *Dosha* vitiation is concentrated on *Amashaya* itself. The primary involvement is vitiated *Vata Dosha* in the *Pakvashaya*. This results in upward movement of *Vata Dosha* leading to absence of passage of flatus and faecal matter. This vitiated *Vata Dosha* is responsible for the further vitiation of *Samana Vata* thereby we are going to find out the Lakshana in *Amashaya* itself.

18 Distension of the abdomen is attributable to delayed gastric emptying i.e. poor motility from the stomach to duodenum. The factors responsible for poor gastric emptying are as follow:

1. The degree of distension of the duodenum
2. The presence of any degree of irritation of the duodenal mucosa
3. The degree of acidity of chyme
4. The degree of osmolality of chyme
5. The presence of certain breakdown products in the chyme, especially breakdown products of proteins and sometimes fats.

If the person is indulging in any of the activities which lead to above phenomena in the stomach, there will be poor gastric and intestinal motility which mimic the features of *Alasaka*.

19 Among the *Asadhya Lakshana* mentioned in *Madhava Nidana*, blackish discolouration of teeth, lips and nails indicate poor blood circulation to the periphery. Altered state of consciousness indicates reduced circulation i.e. syncope. Vomiting, sunken eyes, feeble voice and looseness of joints indicate mechanism of dehydration.

Upadrava of Visuchika²⁰

निद्रानाशोऽरतिः कम्पो मूत्राघातो विसंज्ञता।
अमी छुपद्रवा घोरा विसूच्यां पञ्च दारुणाः॥25॥
प्रायेणाहारवैषम्यादजीर्णं जायते नृणाम्।
तन्मूलो रोगसंघातस्तद्विनाशाद्विनश्यति॥26॥

Ma.Ni. – 6/25,26

- Loss of sleep
- Restlessness
- Involuntary movements
- Retention of urine
- Abnormal sensory perceptions
- All major diseases

Key summary of the chapter

- Ama Pradosha is the spectrum of diseases where we can see acute disturbances of functioning of gastrointestinal tract.
- Visuchika, Alasaka are considered to be manifested in an individual due to Ama Pradosha (accumulation of Ama in the alimentary canal). As per Madhavakara, Visuchika, Alasaka and Vilambika are manifested due to Amajirna, Vishtabdhajirna and Vidagdha Ajirna respectively.
- Visuchika is the disease where cardinal feature observed is abnormal and excessive elimination of morbid Dosha from the upper and lower routes i.e. through mouth and anus simultaneously. Alasaka is the disease where we can observe prolonged stasis of the food in the stomach which can manifest due to the continuous consumption of highly spicy substances. Vilambika is the disease where the primary Dosha is Pitta and in this disease, the person is unable to have elimination of morbid Dosha through oral as well as anal routes.

Self-Assessment: Questions for Reviews

Long essay questions

- Explain Amapradosha in detail

Short essay questions

- Explain Alasaka
- Describe Visuchika
- Elaborate Vilambika

Short answers

- Enumerate Ama Pradosha
- Write the difference between Alasaka and Vilambika
- Define Alasaka
- Write the definition of Visuchika
- Write the Pratyatma Lakshana of Visuchika

MCQs

1. Improper treatment of Amajirna leads to the manifestation of

- a. Ama Pradosha;
- b. Visuchika;
- c. Alasaka;
- d. Vilambika

Answer: (b)

2. Alasaka is manifested in an individual due to improper treatment of

- a. Amajirna;
- b. Vidagdhaajirna;
- c. Vishtabdhajirna;

d. Rasasheshajirna

Answer: (c)

3. As per Charaka, the types of Ama Pradosha are

- a. 1;
- b. 2;
- c. 3;
- d. 4

Answer: (b)

4. The site of disease in case of Alasaka is

- a. Pakvashaya;
- b. Garbhashaya;
- c. Pittashaya;
- d. Amashaya

Answer: (d)

5. Dosha dominance observed in the manifestation of Vilambika is/are

- a. Vata only;
- b. Vata and Pitta only;
- c. Pitta and Kapha only;
- d. Vata and Kapha only

Answer: (d)

6. Murcha Atisaro Vamathuh Pipasa Shulam are the characteristic features of

- a. Alasaka;
- b. Pravahika;
- c. Visuchika;
- d. Vilambika

Answer: (c)

7. The complications of Visuchika are

- a. 3;
- b. 5;
- c. 4;
- d. 6

Answer: (b)

²⁰ If we look the clinical features observed in Upadrava of Visuchika we can thread the each clinical features into the features of circulatory and neurogenic shock.

8. The cardinal feature of Visuchika is pricking type of the body in which part of the body?

- Abdomen;
- Chest;
- All over the body;
- Anus

Answer: (c)

9. As per Madhavakara, the types of Visuchika are

- 2;
- 4;
- 3;
- Types of Visuchika not mentioned by Madhavakara

Answer: (d)

10. The clinical features of Visuchika are resembling the features of

- Acute colitis;
- Acute oesophagitis;
- Acute gastroenteritis;
- Acute encephalitis

Answer: (c)

Chapter resources

- Madhava Nidana – 6th chapter – Agnimandya Ajirna Visuchika Alasaka Vilambika Nidana
- Charaka Samhita Vimana Sthana – 2nd chapter – Trividha Kukshiya Vimana Adhyaya
- Sushruta Samhita Sutra Sthana – 46th chapter – Annapana Vidhi Adhyaya
- Golwalla's medicine for students by Aspi F Golwalla and Sharukh A Golwalla, edited by Milind Y Nadkar, 25th edition, Jaypee Brothers Medical Publishers (P) Ltd, New Delhi, 2017
- The Merck Manual of Diagnosis and Therapy by Robert S Porter and Justin L Kaplan, 19th edition, Merck Sharp & Dohme Corp, New Jersey, 2011
- Chamberlain's symptoms and signs in clinical medicine by Colin Ogilvie and Christopher C Evans, 12th edition, Butterworth-Heinemann Publishers, 1997
- Best & Taylor's physiological basis of medical practice by O P Tandon and Y Tripathi, 13th edition, Wolters Kluwer Health (India) Pvt. Ltd, Gurgaon, 2012
- Textbook of Medical physiology by Arthur C Guyton and John E Hall, 11th edition, Saunders Elsevier publications, Philadelphia, 2006